

HCA Healthcare Transformation Roadmap

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We recommend a gated rollout: prepare the data, local systems and AI controls; test three staff-facing tools in limited settings; expand only after measured results and clinical approval. This document supplies the shared initiative names, visual roadmap, dependencies and phase rationales for the team's RACI and QBR dashboard.

Case and evidence boundary

The group selected HCA Healthcare over Domino's and BayCare. HCA keeps the healthcare context and offers a substantive 2023 Investor Day presentation. Domino's was the alternative for clearer outcome reporting; BayCare offered continuity but a thinner investor-document base. Our evidence window is 2021–2024. This is an analytical reconstruction, not HCA's official roadmap or a claim about its October 2026 deployment status.

Six published initiatives

ID and shared name	What the source supports
I1 Google Cloud data platform	2021 partnership for secure data and analytics; data integration also appears in the Investor Day deck. [S1, S2]
I2 EHR integration and workflow standardization	Our scoped workstream within the published MEDITECH Expanse modernization: validate pilot-site record connections and workflows. Investor Day supports standardization and integration, but does not establish Expanse as a technical requirement for the pilots. [S1, slide 107; S3]
I3 Responsible AI Program	The 2024 Impact Report describes an established program for AI standards, oversight and responsible use. [S4, pp. 14–15]
I4 Timpani nurse scheduling	Named labor-management tool in Investor Day; the Impact Report documents limited deployment and planned expansion. [S1, slide 108; S4, p. 16]
I5 Augmedix clinical documentation	Investor Day names the proof of concept. HCA reported an early pilot with about 75 ER physicians at four hospitals. [S1, slide 109; S3]
I6 Nurse handoff assistant	Shift handoff appears in Investor Day. HCA reported testing a generative-AI handoff tool at UCF Lake Nona Hospital. [S1, slides 110–112; S3]

Counting rule: six distinct initiatives. I4–I6 appear in both Walk and Run because scale follows a pilot; these are not counted as three additional initiatives. Quarter ranges, scopes, gates and targets below are group proposals, not reported HCA commitments.

Visual roadmap and dependencies

Relative plan: Q1–Q3 = months 1–9; Q4–Q6 = months 10–18; Q7–Q10 = months 19–30. Each initiative advances on evidence, not simply because a quarter ends.

Initiative	Crawl Q1–Q3	Walk Q4–Q6	Run Q7–Q10
I1 Google Cloud data platform	Secure pilot data feeds, access controls and quality checks.	Operate and monitor pilot feeds.	Extend validated feeds by site wave.
I2 EHR integration and workflow standardization	Test record access, note write-back and local workflows for I5 and I6.	Maintain validated interfaces; monitor data freshness and errors.	Repeat integration checks for every new site. EHR migration continues in parallel.
I3 Responsible AI Program	Owners, review rules, change control and incident response.	Approve pilots and monitor changes.	Reapprove each new site and model version.
I4 Timpani nurse scheduling	—	I4-W: limited scheduling deployment.	I4-R: enterprise scheduling rollout by eligible site waves.
I5 Augmedix clinical documentation	—	I5-W: limited ER documentation deployment.	I5-R: scale across eligible HCA emergency departments.
I6 Nurse handoff assistant	—	I6-W: one-hospital handoff deployment.	I6-R: scale across eligible inpatient nursing units.

Named capability dependencies

I1 Google Cloud data platform → I5 Augmedix clinical documentation and I6 Nurse handoff assistant: supplies approved, traceable clinical data and secure access. This is our proposed common architecture, not a claim that every vendor uses the same backend.

I2 EHR integration and workflow standardization → I5 Augmedix clinical documentation: validates the interface for writing clinician-approved notes into the record. I2 → I6 Nurse handoff assistant: validates patient matching, access to current records and the local handoff workflow. Without I2, neither pilot can enter integrated production under our proposed release rules. This is a readiness dependency, not a claim that either tool technically requires Expanse.

I3 Responsible AI Program → I4 Timpani nurse scheduling, I5 Augmedix clinical documentation and I6 Nurse handoff assistant: supplies named oversight, human review and controlled model changes.

I4-W → I4-R: proves usable schedules and less preparation work. I5-W → I5-R: proves documentation quality and time benefit. I6-W → I6-R: proves handoff completeness and adoption. Each transition requires the gates on the next page.

Crawl parallelism: I1, I2 and I3 start together. I1 qualifies the secure data platform; I2 tests local record interfaces using test records and existing approved environments; I3 defines oversight. None requires another Crawl initiative to finish. Enterprise Expanse conversion is outside the nine-month readiness scope.

Pilot gates and scale requirements

All thresholds on this page are proposed classroom planning assumptions. Establish a four-week local baseline before each pilot and compare comparable units, workloads and shifts. Clinical leaders must approve the safety criteria before use.

I4 Timpani nurse scheduling

Walk scope: two nursing units in each of three hospitals for a 12-week evaluation. Crawl dependency: I3 oversight. Separate entry requirements: validated workforce and timekeeping feeds, staff skill profiles and approved scheduling rules. No direct dependency on I2 or Expanse is asserted. Readiness outcome: at least 20% less schedule-preparation time than baseline, without worse staffing-rule compliance. At least 80% of eligible nurse managers must actively use the tool.

Run scope: expand I4-W across eligible nursing units in all participating HCA hospital divisions, in waves. Required organizational change: a common scheduling policy, trained local nurse-manager champions, a workforce-data owner and a formal path for local exceptions. A new site enters only after its own readiness checks.

I5 Augmedix clinical documentation

Walk scope: approximately 75 volunteer ER physicians across four hospitals, using the historical pilot's scale as a planning reference. Dependencies: I1 secure data access, I2 tested write-back of approved notes and I3 clinical oversight. Readiness outcome: at least 20% less documentation time per encounter over 12 weeks, with no deterioration in audited note quality and physician approval of every final note.

Run scope: expand I5-W to eligible emergency departments across HCA. Required organizational change: department-level training, a physician owner for note quality, protected training time and ongoing chart audits. Expansion beyond emergency care needs a separate evaluation and is not assumed here.

I6 Nurse handoff assistant

Walk scope: two inpatient units at one hospital for 12 weeks, using the published single-hospital test as a scope reference. Dependencies: I1 clinical data, I2 validated patient matching and current-record access, and I3 review controls. Readiness outcome: at least 15% less handoff-preparation time, no deterioration in audited completeness, and nurse review of every generated handoff.

Run scope: expand I6-W across eligible inpatient nursing units in participating HCA divisions. Required organizational change: a standard handoff checklist, nurse educators, shift-level training and escalation rules. The tool supports, rather than replaces, nurse-to-nurse communication.

Pause and release decisions

An unresolved critical safety or privacy incident pauses the affected pilot or rollout wave. Missed benefit or adoption targets trigger a review before additional funding. Passing a time-saving target never overrides a clinical safety finding. These are proposed governance rules, not claims about HCA's existing policy.

Phase rationales

Crawl rationale

Crawl prepares selected sites for integrated production. I1 provides approved data access, I2 validates record interfaces and workflows, and I3 establishes oversight. These workstreams can begin in parallel; I2 can use test records before the shared platform is complete. Without I2, I5 lacks tested note write-back and I6 lacks validated access to current patient records, so both fail our proposed production gate. Timpani instead needs workforce feeds and scheduling rules. The nine-month foundation covers local readiness, while the broader Expanse migration continues separately. Walk starts only after the relevant checks pass.

Walk rationale

Walk tests whether the tools help real staff in a controlled setting. Timpani, clinical documentation and nurse handoff use the foundation differently, so each needs its own baseline, limited deployment and measured result. A favorable demonstration is not enough. The pilots must show less administrative work without weaker clinical quality or staffing compliance, and users must actually adopt the tools. These checks justify later investment and reveal training or workflow changes while the exposure is still small. Only a successful pilot moves into its corresponding Run rollout.

Run rationale

Run expands proven deployments rather than introducing untested use cases. Each rollout inherits its pilot's evidence, but a new hospital still needs local data, training and safety checks. Enterprise standards create consistency, while an exception process recognizes that hospitals and units differ. Scaling therefore requires organizational changes: local champions, clear ownership, continuing audits and support for staff. We release investment in waves and pause when quality, adoption or benefit targets are missed. This prevents a large installation count from being mistaken for a successful transformation.

What this roadmap does not claim

The proposed phases are a dependency-based interpretation of published initiatives, not HCA's historical calendar. Published pilot or rollout counts demonstrate activity, not proven financial or clinical outcomes. We do not use the unverified September 2026 hospital count, assume cost savings, or describe Run as already complete.

Why the sequence matters

The central risk is scaling a useful-looking tool before local data, staff adoption and clinical review are ready. The practical response is to fund readiness first, collect evidence in a limited setting, and give leaders explicit stop or expand decisions at every gate.

Handoff for the RACI and QBR

Eduardo owns the roadmap and final shared initiative inventory. Alifiya owns the RACI and site editing. Gerald owns the QBR dashboard. The following suggestions connect those deliverables; they do not replace Members B and C's analysis.

Recurring decisions in Alifiya's RACI

RACI row and recurring decision	Accountable role
D1 Budget allocation	EVP and Chief Financial Officer
D2 Architecture standards	SVP and Chief Information Officer
D3 Data governance	EVP and Chief Clinical Officer
D4 Initiative prioritization	EVP and Chief Operating Officer
D5 Vendor and partnership	SVP and Chief Information Officer
D6 Clinical AI release gate	EVP and Chief Clinical Officer
D7 Adoption remediation	Division President

Use realistic, source-checked HCA role titles and exactly one Accountable per decision. These are proposed decision rights, not a description of HCA's actual internal RACI.

Candidate metrics for Gerald

Metric and phase	Type	Proposed target
I1 pilot feeds passing quality checks Crawl	Leading	100% of required feeds before pilot release
I2 I5/I6 sites passing record integration checks Crawl	Leading	100% before integrated production
I3 approved oversight and review plans Crawl	Leading	100% of pilot use cases
I4–I6 active trained users / eligible users Walk	Leading and adoption	At least 80%, reported separately per tool
I4 schedule-preparation minutes Walk	Lagging	At least 20% below matched baseline
I5 documentation minutes per encounter Walk	Lagging	At least 20% below matched baseline
I6 handoff-preparation minutes Walk	Lagging	At least 15% below matched baseline
I4–I6 rollout waves passing all release gates Run	Leading	100% before the next wave

Q6 is the end-of-Walk review at month 18. The dashboard should show measured pilot results when available. If actuals are not public, show "Not measured" until collected, not invented green status. Display safety and quality as release constraints alongside speed metrics. Use these Q6 decisions from the RACI: D6 release I4-W into I4-R or hold; D1 fund the first rollout wave or keep funds on readiness; D7 more training, more time or withdraw an underused tool. The RACI supplies the exact wording and role titles.

Sources and final review

Primary documents were inspected for the claims below. Public evidence supports initiative existence and selected historical scope; proposed targets and dependencies are our analytical work. The group must personally verify the underlying sources before submission, as required by the assignment.

S1 HCA Healthcare Investor Day 2023

November 9, 2023. Printed slides 107–112 cover Expanse, Timpani, Augmedix and shift handoff. Slide 107 supports standard workflows, data and integration within a multi-year Expanse conversion. I2 is our narrower readiness workstream within that published modernization, not a separately named HCA program. The source does not establish an Expanse-only dependency for I4–I6.

[Open the full Investor Day presentation](#)

S2 HCA Healthcare Google Cloud partnership

May 26, 2021. HCA's investor-site announcement documents the secure data and analytics partnership. It establishes the foundation initiative and its date, not completed transformation outcomes.

[Open the 2021 partnership announcement](#)

S3 HCA Healthcare generative AI collaboration

August 29, 2023. HCA's investor-site announcement supplies the historical documentation pilot scope and the nurse handoff test at UCF Lake Nona. Human review remains part of the documented workflow.

[Open the 2023 clinical AI announcement](#)

S4 HCA Healthcare Annual Impact Report 2024

Printed pages 14–16. Responsible AI is described on pages 14–15. Page 16 reports Timpani in nine facilities and another 21 hospitals planned for early 2024. Those figures are dated context, not current deployment counts or evidence of realized savings.

[Open the 2024 Annual Impact Report](#)

Before the group submits

- Confirm the six initiative IDs and names across the roadmap, RACI and dashboard.
- Approve or revise the proposed timeline, pilot scopes, targets and pause rules as a group.
- Check that every Run expansion names its Walk pilot and the organizational change needed.
- Use measured or explicitly illustrative dashboard data. Do not present assumptions as HCA results.
- Personally check the cited pages and rehearse the handoff from A to B to C.

Prepared with AI assistance for drafting and formatting. Source verification and final analytical decisions remain the group's responsibility.